جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : فهيمه احمد ابراهيم شبانه

المحافظة : الجيزه **الحي / المدينة :** بولاق الدكرور **العنوان :** ~~ بولاق الدكرور - الجيزة

رقم البطاقة : 25308061601928 **رقم التليفون :** 01122713344 **تاريخ تسجيل الإستمارة :** 18-10-2026

العمر(تاريخ الميلاد) : 06-08-1953 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(----------)

المقر بما فيه
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فهيمه احمد ابراهيم شبانه
National ID: 25308061601928

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
فهيمه
احمد ابراهيم شباته
٤٤ش ابراهيم رمضان - ناهيا
بولاق الدكرور - الجيزه
٢٨ ١٩ ٠١٦ ٠٦ ٠٨ ٥٣ ٢
IU5019948

DMS


٢٨١٩٠١٦٠٦٠٨٥٣٠٢ ٢٠٢١/٠٣
ربة منزل
أنثى مسلمة متزوجة
محمود احمد محمود طعيمه
١ البطاقة سارية حتى ٢٠٢٨/٠٣/٠٦

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Date Of Submission:

Date Of MDT: 07/28/2026 16:00:32

Medical Report

Patient Name :	فهيمه احمد ابراهيم شبله	Age :	73 Y
Sex :	○ Male◉ Female	Comorbidity :	11335
ID No. :	25308061601928	Telephone :	01112795132
Family History :	DM on diamicron, Cholecystectomy 11 years ago	Marital Status :	○ Married○ Divorced○ Single
Menopausal Status :	عزر	Reason of MDT :	Start ttt as pt in pain
		MDT name::	

Medical Report

Diagnosis : Malignant epithelcid neoplasm (lung)

Notes :

Rt chest wall pain

MRI Dorsal spine: 2/6/2026: Rt paraspinal pleural based at D7 D8 + Right lung lesion

MRI Lumbosacral: spondylodegenerative changes

CT Chest: Multiple right sided pleural based masses - Enlarged right thyroid lobe with a large nodular lesion. Mild mediastinal lymphadenopathy

Pathology of the biopsy from lung mass: Malignant epithelcid neoplasm - Calretinin -ve MOC -ve

History: Finished 7 fractions RT :
Calretinin, MOC 31: -ve
CK 7: strongly +ve
CK 20 -ve
Gata 3 -ve
TTF-1: neg
WT-1: neg
D2-40 minimal +ve cells

Investigations: CT films delivered

Recommendations: PET CT refused.
For CT CAP + Bone scan + MDT to start treatment according the available data

MDT DECISION: For Portacath insertion
Continue workup
To start Alimta - Carboplatin x 4-6 cycles then re-assess for surgery

Speciality	Surgery	Pathology	Radiology

Name: فهيمه احمد ابراهيم شبانه **MR. :** 11336	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

DM on diamicron, Cholecyctectomy 11 years ago. Right chest wall pain. MRI of dorsal spine (2/6/2026) shows right paraspinal pleural-based lesion at D7-D8 and a right lung lesion. MRI of lumbosacral spine shows spondylodegenerative changes. CT chest reveals multiple right-sided pleural-based masses, an enlarged right thyroid lobe with a large nodular lesion, and mild mediastinal lymphadenopathy. Pathology of the lung mass biopsy shows a malignant epithelioid neoplasm; Calretinin negative, MOC negative. Further immunohistochemistry and PET-CT are recommended, with palliative radiotherapy planned.

Finished 7 fractions of radiotherapy. Immunohistochemistry: Calretinin negative, MOC 31 negative, CK7 strongly positive, CK20 negative, Gata3 negative, TTF-1 negative, WT-1 negative, D2-40 minimal positive cells.

PET-CT refused. CT chest, CT abdomen/pelvis, and bone scan are ordered, and a multidisciplinary team (MDT) will initiate treatment based on the available data.

MDT decision: Portacath insertion, continue workup, and start Alimta-Carboplatin (4–6 cycles) followed by reassessment for surgery. The patient did not receive folic acid; folic acid will be started, and the next cycle will be administered.

Pharmacy visit: Initiate Cycle 1 of Alimta 830 mg / Carboplatin AUC 5, weight 63 kg, height 160 cm.

Bone scan on 21/8/2026 shows a solitary active osseous lesion at D7; correlation with CT is needed to clarify its nature. CT chest/abdomen/pelvis on 21/7/2026 shows a few pulmonary nodules, the largest 31 × 17 mm, and a right inguinal lymph node 22 × 16 mm, with no metastatic lesions identified. Pain clinic consultation is recommended.

MDT (7/2026): 1) Portacath insertion, 2) Continue workup, 3) Start Alimta-Carboplatin (4–6 cycles) then reassess for surgery. Cycle 2 of Alimta-Carboplatin has been started.

The patient has experienced attacks of hypotension and fever after each cycle. A 25 % dose reduction is planned for subsequent cycles, with prophylactic G-CSF and 3 days of methotrexate post-treatment. The patient has completed 2 cycles of Alimta/Carboplatin.

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : خالد عبد العزيز محمد كمال

يعتمد ,

ıtient Name	فهيمه احمد ابراهيم شباته	Code Number	YK 297284
:x/Age	Female 72Y	Patient ID	21890
egistered Date	18/06/2026 12:10:46	Referred By Dr.	Interventional Radiology
alidation Date	04/07/2026 12:10:47	Branch	Gustave Roussy
eporting Date	7/4/2026, 2:27:10 PM		

Pathology Report

Pathology Report

Clinical Data: CT chest showing multiple right sided pleural based mass lesions.

Nature of specimen: Right pleural based nodule, Tru-cut biopsy, collected on 18/06/2026.

Gross Examination
Two grey-white tissue cores, each measured 1.2 cm long. Totally submitted in two blocks.

- Technique:
Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: CALRETININ clone (SP65) & MOC31.

Microscopic Examination
Histopathological examination reveals cores of tumor formed of malignant epithelioid cells showing enlarged hyperchromatic nuclei and eosinophilic cytoplasm, arranged in sheets and acini.

IPT (All Controls showed appropriate reactivity):
- Calretinin: Negative.
- MOC31: Negative.

Diagnosis
Right pleural based nodule, Tru-cut biopsy:
- Malignant epithelioid neoplasm.
- Case for further immunophenotyping (D2-40, WT-1, CK7, CK20, TTF-1, GATA-3, etc,,,,).

Signature
Prof. Dr. Maha Guimei.
Dr. Walaa Ibrahem Ghonaim.

Liver Function

atient Name	فهيمه احمد ابراهيم شباته	Code Number	YK 297284
ex/Age	Female 72Y	Patient ID	21890
egistered Date	18/06/2026 12:10:46	Referred By Dr.	Interventional Radiology
alidation Date	04/07/2026 12:10:47	Branch	Gustave Roussy
eporting Date	7/4/2026, 2:27:10 PM		

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مركز سكاي
للأشعة والتحاليل

SKY CENTER
RADIOLOGY & LABS

Patient Name: Faheema Ahmed Ibrahim	Date 04/06/2026
Prof. Dr :	Age : 70 Year(s)
Patient ID: 1922318	Sex F

CT Chest with Intravenous Contrast

Technique

Multidetector CT examination of the chest was performed following intravenous administration of iodinated contrast material. Axial images were obtained with multiplanar reformations in coronal and sagittal planes for optimal anatomical assessment.

Findings

Pleura and Chest Wall:
Multiple right-sided pleural-based mass lesions are identified. The largest lesion is a well-defined, lobulated pleural-based soft tissue mass located in the right paraspinal region at the D7–D8 vertebral level, related to the superior segment of the right lower lobe. It measures approximately **41 × 52 mm** in maximum axial dimensions. The lesion abuts the right paraspinal region and adjacent vertebral bodies without evidence of cortical bone destruction. There is **no definite intraspinal extension**, neural foraminal widening, or invasion of the spinal canal. The remaining pleural-based lesions are smaller, similar in attenuation, and confined to the right hemithorax. No pleural effusion or pleural calcifications are noted.

Lungs:
The lung parenchyma shows preserved overall aeration. Apart from the described pleural-based masses, no definite intraparenchymal pulmonary nodules, masses, or areas of consolidation are identified. No CT evidence of cavitary lesions or interstitial lung disease. The tracheobronchial tree appears patent.

Mediastinum and Lymph Nodes:
Few enlarged mediastinal lymph nodes are noted, the largest being a **retrocaval lymph node measuring approximately 16 × 12 mm**. Other mediastinal lymph nodes are mildly prominent but subcentimetric. No gross mediastinal mass is identified. The esophagus is unremarkable.

Thyroid Gland:
The right thyroid lobe is enlarged and demonstrates a large heterogeneous nodular lesion with **retrosternal extension**. The visualized nodule causes mild mass effect on adjacent mediastinal structures without significant tracheal narrowing or deviation. The left thyroid lobe appears relatively unremarkable within the limits of CT evaluation.

Heart and Pericardium:
The cardiac size and configuration are within normal limits. No pericardial effusion is seen. The visualized cardiac chambers and great vessels show normal contrast opacification.

Patient Name: Faheema Ahmed Ibrahim	Date 04/06/2026
Prof. Dr :	Age : 70 Year(s)
Patient ID: 1922318	Sex F

Great Vessels:
The thoracic aorta and its major branches show normal caliber and course with no evidence of aneurysm, dissection, or significant atherosclerotic changes. The main pulmonary arteries are patent.
Upper Abdomen (Visualized Portions):
The visualized portions of the liver, spleen, pancreas, adrenal glands, and upper poles of both kidneys appear unremarkable, with no focal lesions or abnormal enhancement detected.
Osseous Structures:
The visualized thoracic vertebrae, ribs, clavicles, and sternum show preserved alignment and bone density. No lytic or sclerotic lesions are identified. No evidence of vertebral body collapse or posterior element involvement adjacent to the paraspinal pleural mass.

Impression

- **Multiple right-sided pleural-based mass lesions**, the largest being a well-defined right paraspinal pleural-based mass at the D7–D8 level (41 × 52 mm), **without evidence of intraspinal extension or spinal canal invasion**. Imaging features raise concern for pleural-based neoplastic pathology.
- **Enlarged right thyroid lobe with a large nodular lesion and retrosternal extension**, suggestive of a substernal goiter or thyroid neoplasm.
- **Mild mediastinal lymphadenopathy**, largest retrocaval node measuring 16 × 12 mm, indeterminate and possibly reactive or metastatic depending on the primary pathology.

Correlation with clinical findings and histopathological evaluation is advised.

Much Obliged,
Dr. Amr Moharram, MSc

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمى
The General Organization for Teaching Hospitals
and Institutes
Ahmad Maher Teaching Hospital

To whom it may concern

Mrs / Mr ....فوزية احمد ابراهيم...., 72 years old

female/ male patient presented with diagnosis .....

.....Pelvic mestasis.....

She/ He was referred to us for (radical / adjuvant / Neoadjuvant / palliative) Radiotherapy

She/ He has received (3D/IMRT) radiotherapy to.....

.....Rt. paravertebral mass.....

.....

At Dose : 30 Gy/..10 fx/...2 w .

Additional (Boost) ; yes/ no. at dose .....Gy/.....fx/.....w

(sequential / concomitant)

Started: 12/.7/2026

Ended: 27/.7/2026 With good tolerance .

Head of department
Dr. Hany William

E-mail: Amaher.gothi.com 23914455: ف -23911838- 23913366 : ت 3411شارع بور سعيد — باب الخلق — القاهرة

Patient's Name:	**Faheema Ahmed Ibrahim Shabana**
Date:	**2026-07-29**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Few right lower lobe pleural based pulmonary nodules largest measuring 31 x 17 mm in maximum axial dimensions.
- Tiny right upper lobe calcified pulmonary nodule likely post inflammatory sequelae.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.
- Right thyroid lobe is enlarged in size with retrosternal extension and nodules seen for sonographic assessment.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes. Left renal cortical cyst.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Right inguinal lymph node measuring 22 x 16 mm.

Opinion:

❖ **Few right lower lobe pleural based nodules for PET-CT assessment.**

Dr.sherif Wagdy

Dr.sherif Wagdy

Name : Faheema Ahmed Ibrahim Shabana
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 21-08-2026 06:08
Medical No. : 444294

Tc-99m MDP FOLLOW UP SKELETAL SCINTIGRAPHY

History

73 year-old female patient, complains of bone pain underwent MRI dorsal spine that revealed right paraspinal pleural based mass lesion at DV7-DV8 with additional smaller right lung lesion suspicious of pleuro-pulmonary neoplastic process, true cut biopsy from right pleural based nodule dated 4/7/2026 revealed malignant epithelioid neoplasm and referred for skeletal survey.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- Focal active tracer uptake is noticed at DV7.
- Degenerative features are depicted at different levels of the spine.
- Osteoarthritic changes are noticed around big joints.
- Otherwise rest of the scanned skeleton is unremarkable.

IMPRESSION:-

- Solitary active osseous lesion at DV7 for CT correlation to clarify its nature.
- Rest of the scanned skeleton is unremarkable.

Much obliged
Dr. Alaa Nagy, MD,
Nuclear medicine

NILE DIAGNOSTIC CENTER

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي الترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	فهيمه احمد ابراهيم	**Case No.**	**297284**
Sex/Age	**Female 72 Y**	**Patient No.**	**21890**
Registered Date	**18/6/2026 12:10:46 PM**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	**12/7/2026 12:32:26 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**18/7/2026 03:36:40 PM**	**Pathology No.**	**1923/26**

Pathology Report (Addendum Report)

Clinical Data: CT chest showing multiple right sided pleural based mass lesions.

Nature of specimen: Right pleural based nodule, Tru-cut biopsy, collected on 18/06/2026.

Gross Examination

Two grey-white tissue cores, each measured 1.2 cm long. Totally submitted in two blocks.

- Technique:

Sections were prepared from the paraffin block, processed in Bench-Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: CALRETININ clone (SP65), MOC31, CK7 (SP52), CK20 (SP33), GATA3 (L50-823), TTF-1 (8G7G3/1), D2-40 & WT-1 (6F-H2).

Microscopic Examination

Histopathological examination reveals cores of tumor formed of malignant epithelioid cells showing enlarged hyperchromatic nuclei and eosinophilic cytoplasm, arranged in sheets and acini.

IPT (All Controls showed appropriate reactivity):

- **Calretinin:** Negative.
- **MOC31:** Negative.
- **CK7:** Strongly positive in tumor cells.
- **CK20:** Negative.
- **GATA-3:** Negative.

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	فهيمه احمد ابراهيم	**Case No.**	**297284**
Sex/Age	**Female 72 Y**	**Patient No.**	**21890**
Registered Date	**18/6/2026 12:10:46 PM**	**Referred By Dr.**	**Interventional Radiology**
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- **TTF-1:** Negative.
- **WT-1:** Negative (Showing only membranous staining).
- **D2-40:** Minimal positive cells.

Diagnosis

Right pleural based nodule, Tru-cut biopsy:

- Malignant epithelioid neoplasm.
- Case for further immunophenotyping (PAX-8, Pan-CK, etc,,,,).

Signature

Prof. Dr. Maha Guimei.

Dr. Walaa Ibrahem Ghonaim.

Medical No. : 10871
Patient's Name: Faheema Ahmed Ibrahim Shabana
Date: 2026-10-07
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

جوستاف روسي انترناشونال مصر

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Compared to the previous study dated 29-7-2026, the current study showed:

I- CHEST:

- Regressive course since last study regarding the previously seen few right lower lobe pleural based pulmonary nodules largest measuring 17 x 7.7 mm in maximum axial dimensions.
- Rather stationary course since last study regarding the previously seen two right paratracheal lesions largest measuring 37 x 30 mm ? thyroid nodules ? lymph nodes for sonographic assessment.
- Tiny right upper lobe calcified pulmonary nodule likely post inflammatory sequelae.
- Newly developed right upper lobe small consolidation.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes. Left renal cortical cyst.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Regressive course regarding the size of the previously seen right inguinal lymph node measuring 22 x 9 mm.

Opinion:

- **Regressive course since last study regarding the right lower lobe pulmonary nodules and right inguinal lymph node.**
- **Newly developed right upper lobe consolidation ? inflammatory for clinical and laboratory assessment.**
- **Stationary course regarding the rest of the study.**

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]